



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : جيهان عبد الحليم سيد صالح

العنوان : منسيه ناصر

الحي / المدينة : اوسيم

المحافظة : الجيزه

تاريخ تسجيل الإستماره : 06-10-2026

رقم البطاقة : 27610230101081 رقم التليفون : 01144716559

العمر (تاريخ الميلاد) : 23-10-1976 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: جيهان عبد الحليم سيد صالح National ID: 27610230101081	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

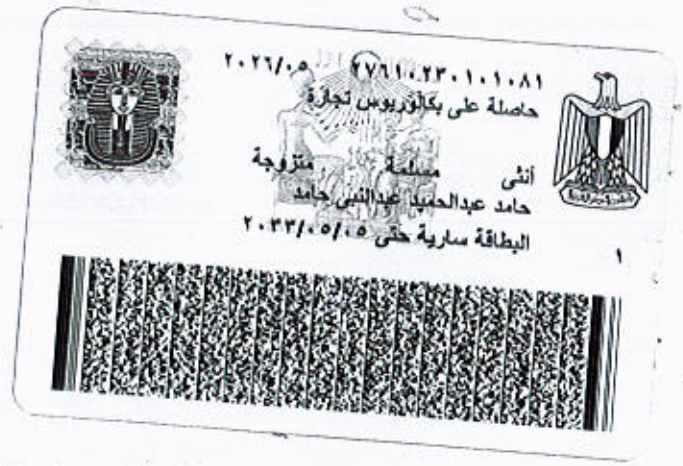
جيهان
عبدالكليم سيد صالح
٣ ح الفيومية - من الورشه
منشيه ناصر - القاهره

١٩٧٦/١٠/٢٣ ٠١٠١٠ ٨١

LT1363939

DMS

تد عيسى



Name: جيهان عبدالحليم سيد			
MR. : 1568			
Detailed Medical Report			
		السيد	
تحيّة طيبة وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is receiving Femara 2.5 mg daily with supportive care, continuing a 3-year Femara regimen and a 6-month Zometa course. The next clinic visit is scheduled for 11/8/2026.			
A neck ultrasound performed in July 2026 identified a right thyroid nodule classified as TIRADS 5, warranting histopathological correlation. A decision to obtain a tissue sample has been made and the order has been issued.			
تحريراً في ,			
التاريخ :		20-09-2026	
الوقت :		نهى سعيد علي محمد	
الطبيب المعالج :		يعتمد ,	

03/18/2026

MDT FORM

Name:	Jihan Abd Alhalim El-Sayed .	MRN:	1568
Age:	49 Y	ID:	27610230101081
Menopausal Sta	Post	Phone:	01144716559
NRS SCORE:	PS ECOG 0	Marital Status:	
BMI:		Family History:	ve
CO Morbidity:	DM , IHD	Reason Of MDT:	

Clinical Data:

BX: IDC G II

ER: 8/8 **PR:** 8/8 **HER2:** +2 not ar **KI67:** 38%

R

L

Sonomammography:

Mets Workup:

History:

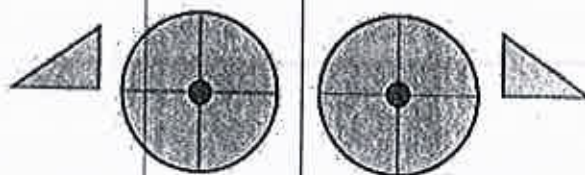
3-2023 >> upfront BCS + SLNB >> pT2 Pn0 >> adj taxotere , endoxan * 4 cycles >> PORT >> on tam since 7/2023
last E2 level (12-2025) : 6.260

Recommendations:

after Dr khaled consultation >> shift to AI

MDT Outcome & Plan

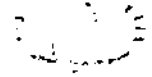
for Femara 3y
Zometa/6m



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ شى حن					



Patient Name	: الأستاذة / جيهان عبدالحليم سيد صالح	Request No.	: 13001262
Patient No.	: 27610230101081	Request Date	: 07/07/2024
Path No.	: C-224/24	Verification Date	: 22/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right thyroid nodule (TIRADS 2).

Nature of Specimen: Cytology.

Gross Examination:

- Four alcohol fixed and unstained smear slides were received.

Microscopic Examination:

Smears examined reveal fairly cellular smear showing bland looking and degenerative follicular cells arranged singly and in follicles on a hemorrhagic background. No evidence of papillae or nuclear anaplasia. No evidence of cytologic malignancy.

Diagnosis:

Right thyroid nodule, Cytology:

Cytologic features of follicular lesion with degenerative changes suggest benign lesion, Bethesda II, for correlation with the clinical and radiological findings.

Signature

Prof. Dr. Amal Ahmed Hareedy.

Dr. Heba Hasan.

Dr. Rufaida abdullah.

Verified by :

Dr.Rofayda Abdallah

Printing Date : 28/07/2024 10:39:33

Page 1 of 1

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr-El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / جيهان عبدالحليم سود صالح	Request No.	: 13000145
Patient No.	: 27610230101081	Request Date	: 06/02/2023
Path No.	: 354/2023	Verification Date	: 26/02/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data:

Right breast mass lesion

Nature of the specimen:

Tru cut biopsy collected 6-2-2023 (12:50PM)

Gross Examination:

Multiple greyish white tissue cores up to 1 cm . Totally submitted

Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2 ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Angioinvasion: No
- Ductal carcinoma in situ: No

*PT:

ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)

PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)

Her-2: Moderate incomplete membrane reaction in >10% of tumor cells (score 2+) , equivocal

Ki 67: 30 % nuclear reaction in tumor cells

Average Her2 copy No : 1.3

Her2/CEP 17 ratio : 0.77

SISH : Not amplified

Diagnosis:

Right breast mass lesion , Tru cut biopsy:

Verified by .
dr.esraa magdy



Patient Name	: الأستاذ / جيهان عبدالحليم سيد صالح	Request No.	: 13000145
Patient No.	: 27610230101081	Request Date	: 06/02/2023
Path No.	: 354/2023	Verification Date	: 26/02/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

• Invasive duct carcinoma grade II
-IPT:
ER: Positive (score 8/8).
PR: Positive (score 8/8).
HER-2: Equivocal (score 2+)
Ki67: Positive in 30% of tumor cells.
SISH : Not amplified

Prof.Dr Gamal Abdelatty
Dr.Heba Hassan

Dr.Esraa Magdy

Verified by :
dr.esraa magdy



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذ / جيهان عبدالحليم سيد صالح	Request No.	: 13003138
Patient No.	: 27610230101081	Request Date	: 27/03/2023
Path No.	: 970/23	Verification Date	: 05/04/2023
Sex	: Female	Financial Category	: نفقة النولة - اورام دار السلام
Age	: 46 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

CLINICAL DATA:

Right breast cancer, upfront

NATURE OF SPECIMEN:

Right BCS & SLN specimens

GROSS EXAMINATION:

- 1- Right BCS specimen measured 10x7x3 cm , with covering skin ellipse measured 8.5x3 cm
- Tumor extent
 - o Number: A firm grayish white tumor
 - o Size: 1.5x1x0.5 cm
 - o Skin involvement: Free grossly
 - Distance from margins: 2cm from deep margin (least margin)
 - Right SLN: A fibro fatty tissue piece measured 5x2.5 cm & entangling 3 nodules up to 2x1cm.

MICROSCOPIC EXAMINATION:

Right breast ; BCS & SLN biopsy :

- Histological type: Invasive duct carcinoma with extensive stromal lymphocytic reaction
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Ductal carcinoma in situ: Major component
- Skin involvement: Free
- Lymph node involvement: Free SLNs (0/3)
- Angio invasion: No
- Perineural invasion: No
- Resection Margins: Free all resected margins

DIAGNOSIS:

Verified by :

Dr. Hebatallah Mostafa

Printing Date : 11/04/2023 12:47:47

Page 1 of 2

Printing By : .marwa mohamed youss

رقعة رقم (4) - معمل الباثولوجي - مبنى رقم (1087) - مستشفى عين شمس التخصصي - القاهرة - مصر
Room No. (4) - First Floor Building No. 1087 - Nile Clinics - Shousha El Qandous Centre, Egypt



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي



Patient Name	: الأستاذة / جيهان عبدالحليم سيد صالح	Request No.	: 13003138
Patient No.	: 27610230101081	Request Date	: 27/03/2023
Path No.	: 970/23	Verification Date	: 05/04/2023
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 46 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Right breast ; WLE & SLN Biopsy :
-Invasive ductal carcinoma, grade II, with major component of DCIs
-Free all resected margin
-Free SLNs (0/3)
-Stage : pT1, pN0

Signature
Prof. Iman Gouda
Dr.Hebatallah Mostafa

Verified by :
Dr.Hebatallah Mostafa

Printing Date : 11/04/2023 12:47:47

Page 2 of 2

Printing By : .marwa mohamed youss

غرفة رقم (4) - كادر رقم (587) - مستشفى القصر - مبنى طباعة - طابق 4 - مبنى
Room No. (4) - First Floor-Building No. 587 - Nile Corniche - Main El Gouda - Cairo, Egypt



Patient Name	: الأستاذ / جيهان عبدالحليم سيد صالح	Request No.	: 13003138
Patient No.	: 27610230101081	Request Date	: 27/03/2023
Path No.	: 970/23	Verification Date	: 05/04/2023
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 46 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Right breast ; WLE & SLN Biopsy :

- Invasive ductal carcinoma, grade II, with major component of DCIs
- Free all resected margin
- Free SLNs (0/3)
- Stage : pT1, pN0

Signature
Prof. Iman Gouda
Dr.Hebatallah Mostafa

Verified by :
Dr.Hebatallah Mostafa

Patient Name	: الأستاذة / جيهان عبدالحليم سيد صالح	Request No.	: 13000145
Patient No.	: 27610230101081	Request Date	: 06/02/2023
Path No.	: 354/2023	Verification Date	: 19/02/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة النولة
Age	: 46 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Right breast mass lesion

Nature of the specimen:

Tru cut biopsy collected 6-2-2023 (12:50PM)

Gross Examination:

Multiple greyish white tissue cores up to 1 cm . Totally submitted

Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:

- **Histological type:** Invasive duct carcinoma
- **Grade (differentiation+ nuclear pleomorphism + mitosis):** G II(3+2+1)
- **Angioinvasion:** No
- **Ductal carcinoma in situ:** No

***IPT:**

ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)

PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)

Her-2: Moderate incomplete membrane reaction in >10% of tumor cells (score 2+) , equivocal

Ki 67: 30 % nuclear reaction in tumor cells

Diagnosis:

Right breast mass lesion , Tru cut biopsy:

- Invasive duct carcinoma grade II

-IPT:

ER: Positive (score 8/8).

Verified by :

dr.esraa magdy

Printing Date : 26/02/2023 11:58:20

Page 1 of 2

Printed By : .marwa mohamed youss



Patient Name الأستاذ / جيهان عبدالحليم سيد صالح
Patient No. 27610230101081
Path No. 354/2023
Sex Female
Age 46 Y
Referring Doc.

Request No. 13000145
Request Date 06/02/2023
Verification Date 19/02/2023
Financial Category اورام دار السلام - نفقة التولية
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

PR: Positive (score 8/8).
HER-2: Equivocal (score 2+) , for SISH.
Ki67: Positive in 30% of tumor cells.

Prof.Dr.Gamal Abdelatty
Dr.Heba Hassan

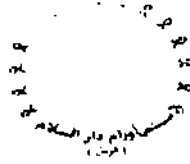
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

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Page 2 of 2

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Patient Name: Jihan Abdel Haleem Sayed Saleh	Date : 16/01/2024
Exam : MAMMOGRAPHY BOTH	Age : 47 Year(s)
Medical ID: 688/2023	Pat. ID: 29424

**BILATERAL BREAST MAMMOGRAPHY AND HIGH
RESOLUTION ULTRASOUND**

Indication:

Post right breast conserving therapy

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)

Right breast:

- Operative bed minimal parenchymal distortion with no sizable mass
- No suspicious mass lesions, micro calcifications.
- No sizable collection.
- Normal skin thickness and contour.
- No regional lymphadenopathy

Left Breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post right BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Best regards,

Dr. Amal Ibrahim

Dr. Amira H Radwan MD,
Breast Imaging consultant,

NCI, Cairo University

16/01/2024 3:29 PM



Patient Name: Jihan Abdel Haleem Sayed Saleh	Date : 29/12/2024
Exam : MAMMOGRAPHY BOTH	Age : 48 Year(s)
Medical ID: 688/2023	Pat. ID: 29424

**BILATERAL BREAST MAMMOGRAPHY AND HIGH
RESOLUTION ULTRASOUND**

Indication:

Post right breast conserving therapy

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

Scattered fibro glandular density (ACR B)

Right breast:

Operative bed minimal parenchymal distortion with no sizable mass

No suspicious mass lesions, micro calcifications.

No sizable collection.

Normal skin thickness and contour.

No regional lymphadenopathy

Left Breast:

No suspicious micro calcifications or architectural distortion detected.

No spiculated mass

Normal retro-areola ducts

Normal skin thickness, and nipple-areola complex

Nonspecific axillary LN.

Opinion:

Post right BCS status showing no evidence of recurrence (BIRADS 2)

Normal study of the left breast (BIRADS 1)

Signature Dr. Wafaa Ali

29/12/2024 2:07 PM

أ. فرحان موصى و. سنوار
ع. الندي



Date : 22/06/2024	Patient Name: Jihan Abdel Haleem Sayed Saleh
Age : 47 Year(s)	Exam : L.S Neck
Pat. ID: 29424	Medical ID: 688/2023

Neck Ultrasonography

- The right and left lobes and isthmus of thyroid gland are enlarged showing multiple spongiform nodules, the largest seen at right lobe measures 2x 1.5 cm .
- Normal vascularity at CCDI.
- No solid nodules detected.
- The gland moves freely with deglutition.
- The trachea is centralized.
- Both carotid sheaths are free.
- No pathological cervical lymph nodes .

Conclusions

- Bilateral spongiform thyroid nodules (TI-RADS 2)

Best Regards,
Dr Shahid M. Fouad, MSc, FRCR

Not Approved



Patient Name: Jihan Abd Al Haleem Sayed Saleh	Date : 02/03/2023
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID:	Pat. ID: 29425

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- Right breast UIQ asymmetrical density is seen.
- Prominent right axillary lymph nodes are seen.
- Prominent subcentimetric right supra-clavicular lymph nodes.
- No sizable pulmonary nodules.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Heterogenous texture of thyroid gland for US correlation.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus is seen.
- Left ovarian simple cysts the largest one measures 2.5 x 2.2 cm likely functional
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

Patient Name: Jihan Abd Al Haleem Sayed Saleh	Date : 02/03/2023
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID:	Pat. ID: 29425

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***No pulmonary, hepatic or osseous metastatic deposits.***
- ***Bulky uterus for US assessment.***

Dr. Wafaa Ali

~~Signature Prof. Menna Talla-Hanafy~~

~~04/03/2023 02:07~~

Patient Name: Jihan Abdel Haleem Sayed Saleh	Date : 27/07/2025
Exam : ultrasound TRANS VAGINAL	Age : 45 Year(s)
Medical ID: 688/2023	Pat. ID: 29424

Trans-vaginal ultrasound scanning revealed: -

○ **Uterus: -**

- *AVF, average size uterus measuring about: 8x4x4cm in its longitudinal, transverse & AP diameters respectively.*
- *The myometrium: shows homogenous texture with no focal lesion*
- *The endometrial line is 6mm.*

○ **Ovaries: -**

- *Both ovaries are average size*
- *Left adnexal elongated cystic lesion not separable from the left ovary measuring 30x16mm ??hydrosalpinx? ovarian simple cyst*

- *No free fluid in the Douglas pouch.*

Conclusion:

Left adnexal elongated cystic lesion not separable from the left ovary??hydrosalpinx? ovarian simple cyst for follow up

Signature Dr . Nermien Ramadan
PM 1:10 2025/07/27

PATIENT NAME: GEHAN ABD EL HALEEM SAYED SALEH

GENDER: Female

DATE : 03-Jun-2025 02:00 PM

PATIENT ID: 10125264430

AGE: 48 y

ORDER: 3402403

Whole body [18F] FDG PET-CT Scan

CLINICAL HISTORY:

48-year old female patient, presented with right breast cancer, underwent CBS and received CTH, RTH ended in 2023, currently on hormonal therapy.

Reason for the study: detection of FDG avid neoplastic/metastatic lesions.

TECHNIQUE:

- **Radiopharmaceutical:** 18F-FDG, intravenously.
- **Time from 18F-FDG injection to scan:** 60 min
- **PET/CT images** were acquired from the skull vault through the upper thighs & **CT images** were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and sagittal planes.
- **Patient's weight:** 84 Kg - **Patient's height:** 155 cm - **Bl. Gluc. Level:** 103 mg/dL
- **Additional notes:** SUV max normalized to body weight was used.
- **Reference hepatic activity (SUV max):-** 2.7

FINDINGS:

Breast and axillary regions:

- **Status post-right conservative breast surgery with operative bed soft tissue sheets, likely post-therapeutic sequel yet no sizable FDG avid operative bed residual neoplastic lesions.**
- **No FDG avid right axillary lymph nodes. Only tiny non FDG avid few right axillary lymph nodes reaching 9 mm likely reactive.**
- **No FDG avid left breast lesions or left axillary lymph nodes.**

Head and Neck:

- **Normal diffuse brain activity with no hypermetabolic brain lesions.**
- **Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx.**
- **Physiologic FDG uptake is seen in submandibular and parotid salivary glands.**
- **MNG with small right thyroid lobe calcific focus yet no focal FDG avid lesions for U/S correlation.**
- **FDG avid bilateral upper and right lower cervical lymph nodes reaching**

PATIENT: GEHAN ABD EL HALEEM SAYED SALEH, 10125264430, Female, 48 y

Report version 3, 04-Jun-2025 01:01 PM

Page 1 of 2

1.5 cm with SUVmax~3.9 at right level II, likely reactive for follow up.

Chest:

- Non FDG avid mediastinal retro-caval lesion of low attenuation value measuring 3x2 cm likely of benign nature mostly prominent pericardial recess.
- Non FDG avid anterior mediastinal soft tissue density, likely thymic remnant.
- No FDG avid mediastinal lymph nodes.
- No FDG avid lung nodules or masses detected.
- Right middle lobe sub-pleural lung reticulations, likely post-RTH changes.
- No metabolically active pericardial or pleural lesions detected.

Abdomen and Pelvis:

- Physiologic diffuse FDG uptake is seen over enlarged liver. No FDG avid focal lesions detected.
- Physiologic diffuse FDG uptake is seen over average sized spleen with homogenous parenchymal texture and no FDG avid focal lesions seen. Small splenule is noted.
- Normal appearance of both kidneys with no stones or back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.
- Normal appearance of the pancreas and both supra-renal with no FDG avid lesions detected.
- No evidence of ascites or abdominal collections noted.
- No FDG avid abdominal or pelvic nodal lesions.

Musculoskeletal:

- Bilateral sacro-ilitis.
- Diffuse FDG uptake is noted over right para-vertebral muscles opposite lower dorsal vertebrae of SUVmax~5.2, likely spasm related for clinical correlation.
- No focal active blastic or lytic bone lesions detected.

Conclusion:

FDG PET/CT study for a known case of managed right breast cancer revealed;

- Evidence of right BCS with operative bed changes yet no definite FDG avid residual/recurrent neoplastic lesions.
- No definite FDG avid regional or distant metastatic lesions.

Reported By,

Dr. Mohamed Abdeltawab ,MSc

Dr. Mohamed Mohsen, MD

Prof. Huda Fathy, Nuclear Medicine Consultant

04-Jun-2025 01:01 PM

PATIENT: GEHAN ABD EL HALEEM SAYED SALEH, 10125264430, Female, 48 y

Report version 3, 04-Jun-2025 01:01 PM

Page 2 of 2



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PET Scan

[[DOCTYPE:PET_SCAN]]

Patient Name	جيهان عبد الحليم سيد	Case No.	359005
Sex/Age	Female 49 Y	Patient No.	26074
Registered Date	19/8/2026 08:34:42 AM	Referred By Dr.	Interventional Radiology
Validation Date	16/9/2026 03:07:09 PM	Branch	Gustave Roussy
Reporting Date	20/9/2026 01:44:55 PM	Pathology No.	C-197/26

Cytology Report

Clinical Data: Neck U/S showing right thyroid lobe nodule (TIRADS IV).

Nature of Specimen: Right thyroid lobe nodule, U/S guided FNA, Cytology, Collected on 19/08/2026.

Gross Examination:

Four alcohol fixed smears were received & then they were (H&E) stained.

Microscopic examination

- Adequacy: Adequate smears.
- Cellularity: Hypocellular.
- Colloid: Present, abundant & thick.
- Follicular cells: Few, forming macro-follicles.
- Nuclear enlargement: Absent.
- Nuclear overlapping: Absent.
- Nuclear grooving: Absent.
- Psammoma body: Absent.
- Other cellular elements: Numerous foamy & hemosiderin laden macrophages.

Diagnosis

Right thyroid lobe nodule, U/S guided FNA, Cytology:

- Findings are consistent with benign colloid nodule, Bethesda category II.

Comment: The BETHESDA System for reporting thyroid cytopathology: Diagnostic categories:

- I. Non-diagnostic or Unsatisfactory.
- II. Benign. Consistent with a benign follicular nodule (includes adenomatoid nodule, colloid nodule, etc.).



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